

國泰綜合醫院

ADMISSION NOTE

病歷號碼: 0028018818 姓名: 祝鳳英 身分證號: A290073807
 床號: 72801 年齡: 50 性別: 女 出生日期: 1975/10/02
 科別: 復健科 主治醫師: 黃雅鈴 入院日期: 2026/07/08
 文件編號: 00100017202607211701002524

Source of information

Patient、Patient's husband

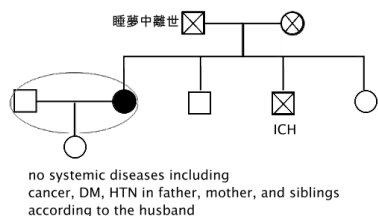
General Data

1. Gender: Female / Age: 50歲
2. Marital Status: +
3. Education: high school
4. Occupation: restaurant
5. Date of history taking: 2026/07/08
6. BH: 155.0 cm, BW: 46.0 kg, BMI: 19.15

Personal History

Cigarette smoking: -
 Alcohol consumption: -
 Betel nut chewing: -
 Travel history: -
 Occupation history: -
 Contact history: -
 Cluster history: -
 Special environmental exposure: -
 History of venereal disease: -
 Pet animal history: -

Family history: brother ICH; no systemic diseases including cancer, DM, HTN with father, mother, and siblings according to the husband



Allergy History

1. Drug: -
2. Food: -

Drug History

Mosapride 5mg 【Mopride F.C tab 5mg】 劑量1 粒 頻率BIDPC PO (自備)
 Tolperisone 150mg 劑量1 粒 頻率TIDPC PO (自備) Titrate from Original: 1# BIDPC
 Amlodipine 5mg 【Norvasc 5mg】 劑量0.5 粒 頻率QDPC PO (自備)
 Amylase 10mg 劑量1 粒 頻率TIDPC PO (自備)
 Bisoprolol 1.25mg 【Concor 1.25mg】 劑量1 粒 頻率BIDPC PO (自備)
 Sertraline 50mg 【Zoloft 50mg】 劑量1 粒 頻率HS PO (自備)
 Quetiapine 25 mg 【Utapine F.C. tab】 劑量0.5 粒 頻率HS PO (自備) titrate from original 0.25# HS

Past History

1. DM:
-
2. HTN:
- +
3. Other systemic disease:

Anterior com aneurysm rupture with extensive SAH (subarachnoid hemorrhage (hematoma)) and hydrocephalus s/p (status post) 1.EVD (Extraventricular Drainage) with ICP (intracranial pressure) monitoring

insertion (via right Kocher's point) (Codman ICP (intracranial pressure)) on 2025/03/15 ; 2. Trans-arterial embolization of Acom Aneurysm (simple coiling), sonography guided femoral puncture + DSA (Bilateral ICA (internal carotid artery), Left VA (visual acuity), Right femoral artery (extremities) on 2026/03/15, with bilateral hemiplegia, post-stroke spasticity, and cognitive impairment

Hypertension

4. Operation history:

As above

5. Vaccine records:

Nil

Chief Complaint

Conscious change on 03/14 night

Present Illness

This 50-year-old woman was previously independent in all activities of daily living and was a community ambulator without an assistive device. She had been in her usual state of health until 2026/03/14, when she suddenly collapsed at home following the onset of a severe headache. She was found unconscious and brought to the CGH emergency department in a comatose state (E1M4V1). Emergent brain CT revealed diffuse subarachnoid hemorrhage secondary to rupture of an anterior communicating artery (Acom) aneurysm. An emergent external ventricular drain (EVD) and intracranial pressure monitor were inserted through the right Kocher's point on 03/15. She subsequently underwent endovascular transarterial embolization of the Acom aneurysm with simple coiling via femoral access. Postoperatively, she was admitted to the SICU for blood pressure control and neuroprotection. Follow-up brain CT on 03/26 demonstrated bilateral frontoparietal SAH with intraventricular hemorrhage in the lateral ventricles. She was transferred to the general ward on 04/02.

During the hospitalization, recurrent fever developed beginning on 03/19. Cerebrospinal fluid and EVD tip cultures yielded *Staphylococcus* species, raising suspicion for a device-related infection. After consultation with the infectious disease specialist, intravenous vancomycin and ceftriaxone were initiated on 04/02. Her fever gradually subsided over the following weeks. However, on 04/17, widespread symmetric blanchable erythematous maculopapular rashes developed over the trunk and proximal lower extremities. A drug eruption was suspected, and oral corticosteroids and antihistamines were initiated. Vancomycin was discontinued on 04/18, and hydrocortisone was gradually tapered beginning on 04/24. She was subsequently transferred to the Sijhih neurosurgical ward and then to the PM&R ward on 04/30 and 05/06, respectively. During her admission at Sijhih, the Foley catheter was successfully removed after bladder training on 05/19. She was later transferred to Renai Hospital for rehabilitation, where she developed severe spasticity of both lower extremities, which was treated with tolperisone 150 mg twice daily with partial improvement.

Neurologically, her consciousness gradually improved from the initial comatose state to E4M6V3 by the time of transfer to the general ward on 04/02. Manual muscle testing showed grade 1 strength in both upper extremities and grade 2 strength in both lower extremities. By 05/06, her Brunnstrom stages had improved to stage III in the right upper extremity and right lower extremity. She was able to consistently follow simple commands. By 07/07, her Brunnstrom stages had further improved to right upper extremity (proximal/distal: IV/V), right lower extremity (IV), left upper extremity (proximal/distal: V/V), and left lower extremity (V). MMT demonstrated grade 2 strength in the right upper extremity, grade 3?? strength in the left upper extremity, grade 2?? strength in both hip muscles, and grade 3?? strength in both ankles. In terms of language function, she was able to follow two-step commands and communicate using short sentences. Functionally, she remained dependent for wheelchair propulsion and was still largely dependent for overall daily activities. Given the above neurological deficits, she was transferred to the CGH PM&R ward for intensive rehabilitation on 07/08.

[Premorbid status]

Living environment: Resided with her husband in a residence requiring negotiation of two flights of stairs.

Occupation: Restaurant front-of-house staff.

Education: High school graduate.

Ambulation: Community ambulator without an assistive device.

ADLs: Independent.

Primary caregiver: Husband.

Review of Systems

General: body weight change(-), weakness(+), fatigue(-), fever(-), chills(-)

Skin: rashes(-), lumps(-), sores(-), itching(-), dryness(-), color change(-), change in hair or nails(-)

Head: headache(-), head injury(-)

Eye: vision(-), glasses or contact lenses(-), pain(-), redness(-), excessive tearing(-), double vision(-), glaucoma(-), cataract(-)

Nose and sinuses: frequent colds(-), nasal stuffiness(-), discharge(-), itching hay fever(-), nose bleeds(-), sinus trouble(-)

Neck: lumps in the neck(-), swollen glands(-), goiter(-), pain or stiffness(-)

Breast: lumps(-), pain or discomfort(-), nipple discharge(-), self-exam(-)

Cardiovascular: chest pain(-), high blood pressure(-), rheumatic fever(-), heart murmurs(-), orthopnea(-), paroxysmal nocturnal dyspnea(-), edema(-)

Respiratory system: cough(-), sputum(-), hemoptysis(-), wheezing(-), asthma(-), bronchitis(-), emphysema(-), pneumonia(-), tuberculosis(-), pleurisy(-)

Endocrine: thyroid trouble(-), heat or cold intolerance(-), excessive sweating(-), diabetes(-), excessive thirst or hunger(-), polyuria(-)

Hematologic: anemia(-), easy bruising or bleeding(-), post transfusion reaction(-)

Lymphatic: lymphadenopathy: location(-), size(-), character(-)

Gastrointestinal: trouble swallowing(-), heart burn(-), appetite(-), nausea(-), vomiting(-), regurgitation(-), vomiting of blood(-), indigestion(-), color and size of stools(-), change in bowel habits(-), rectal bleeding or tarry stool(-), hemorrhoids(-), constipation(-), diarrhea(-), abdominal pain(-), food intolerance(-), hepatitis(-), jaundice(-), liver or gallbladder disease(-), excessive belching or passing of gas(-)

Genitourinary: frequent urination(-), polyuria(-), nocturia(-), burning or pain on urination(-), hematuria(-), urgency(-), reduced caliber or force of the urinary stream(-), hesitancy(-), incontinence(-), urinary tract infection(-), stones(-)

Musculoskeletal: muscle or joint pain(-), stiffness(-), arthritis(-), gout(-), backache(-)

Rheumatologic: hair loss(-), malar rash(-), discoid rash(-), oral ulcer(-), arthralgia(-), dry eye(-), dry mouth(-)

Neurologic: consciousness change(-), fainting(-), blackouts(-), seizures(-), weakness(-), paralysis(+) bilateral hemiplegia, numbness(-), tingling(-), tremors or other involuntary movements(-)

Psychiatric: nervousness(-), tension(-), mood change including depression(-), memory loss(-), auditory hallucination(-), visual hallucination(-), involuntary movement(-)

Others:

Physical Examination

Vital signs: stable

General appearance: fair spirit

Consciousness: GCS (Glasgow Coma Scale): E 4 M 6 V 5

HEENT: Head: trauma(-), facial swelling(-), puffy face(-)

Eye: conjunctiva: anemic(-) sclera: icteric(-)

Ears: otorrhea(-)

Nose: rhinorrhea(-), discharge(-)

Throat: lip cyanosis(-), oral ulcer(-), injected(-), tonsil enlargement(-), tonsil pus coating(-)

Neck: supple, thyroid enlargement(-), jugular vein engorgement(-), carotid bruit(-), lymphadenopathy(-)

Chest: Inspection: accessory muscle respiration(-), paradoxical movement(-)

Palpation: tender point(-), palpable mass(-), tactile fremitus : symmetrical

Percussion: abnormal dullness(-)

Auscultation: clear breathing sound, rales(-), rhonchi(-), wheezing(-)

Heart: Inspection: visible apical impulse

Palpation: thrill(-), heave(-), palpable apical impulse at 5th intercostal space and left mid-clavicular line

Percussion: dullness between 2nd to 5th intercostal space

Auscultation: regular heart beats, ectopic beats(-), S3(-), S4(-), murmur(-), gallop rhythm(-)

Abdomen: Inspection: distension(-), Cullen's sign(-), Turner sign(-), spider nevi(-), superficial vein engorgement(-)

Palpation: tenderness(-), muscle guarding(-), rebounding pain(-),

palpable mass(-), hepatomegaly(-), splenomegaly(-), Murphy's sign(-)

Percussion: shifting dullness(-), fluid wave(-), tympanic(-)

Auscultation: normoactive bowel sound ,bruit(-)

Flank: knocking pain L(-) / R - , pressure sore(-)

EXT: freely movable , pitting edema(-) , joint swelling(-) , joint tenderness(-) , Janeway lesion(-) , Osler's nodules(-)

Skin: skin turgor(-) , rash(-) , petechiae(-) , gangrene(-)

Lymph node: neck(-) , supraclavicle(-) , axillary(-) , inguina(-)

Digital Rectal examination:

Peripheral vascular:

(1)Radial a. --> Right side (++) Left side (++)

(2)Brachial a. --> Right side (++) Left side (++)

(3)Femoral a. --> Right side (++) Left side(++)

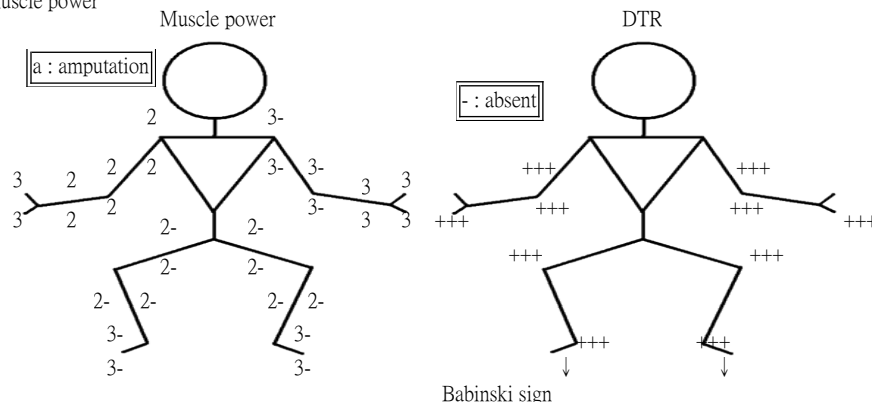
(4)Popliteal a. --> Right side (++) Left side (++)

(5)Tibial a. --> Right side (++) Left side (++)

(6)Dorsalis pedis a. --> Right side (++) Left side (++)

Others: [Consciousness & Cognition] Consciousness: E4M6V4, awake but disoriented Appearance: fair spirit Response: cooperative Mood: stable [Mood Assessment] PHQ-2: 1. Little interest: 0 2. Feeling down: 0 [4AT: 8] 1. Alertness: 0 2. AMT4 (Age, date of birth, place (name of the hospital or building current year): 2 mistakes 3. Attention (serial 3s): 2 unable to test 4. Acute change: 4 [MMSE] 10/30 Time(5)/Place(5): 1/0 Registration(3)/Recall(3): Attention and calculation(5): 0 Naming(2)/Repeat(1)/Reading(1)/Writing(1): 1/1/1/0 3-step command(3)/Copy drawing(1): 3/0 [Speech & Language language: Mandarin -LAST-A score: 11/15 Expression index: 6/8 Naming 3, Repetition 2, Auto speech 1 Receptive index: 5/7 Picture 2, Verbal 3 -/ comprehension: impaired Object/Body part: correct -Expression: fluent, spontaneous Volume: decreased Sentence complexity: short sentence Stereo -Reading aloud: intact, Reading comprehension: intact -Dysgraphia: +, Alexia: - Impression: Cognitive impairment [Swallowing] Feeding: normal d without choking FOIS score: 6 RSST: 5 times Cough ability: functional Tongue movement: full 3 mL Swallow Trial: - Oral stasis: -, Drooling: -, Pr spillage: - - Time to larynx elevation: 1—3s - Larynx elevation: full - Choking: - - Wet voice: - - Multiple swallows: - [Cranial Nerves] CN II: visual intact, visual field intact CN III/IV/VI: pupils 3/3 mm, light reflex R/L: +/+, ptosis: -, EOM: full CN V: facial sensation , mastication intact CN VII: palsy - CN VIII: hearing symmetrical, VOR intact CN IX/X: palate elevation symmetrical, uvula midline, gag reflex preserved CN XI: SCM intact, trapezius impaired CN XII: tongue protrusion midline [Motor Status] Brunnstrom: RUE(P/D): IV/V LE: IV (may be underestimated due to spasticity (P/D): V/V LE: IV (may be underestimated due to spasticity) MMT: RUE(P/D): 2 /3, RLE(P/D): 2-/3- LUE(P/D): 3-/3, LLE(P/D): 2-/3- DTR(R/L) flexor: +++/+++ Elbow extensor: +++ /+++ Brachialis: +++ /+++ Knee extensor: +++/+++ Ankle plantarflexor: +++/+++ no clonus on exam Spastic (MAS): (R/L) Elbow flexor: 1/1 Wrist flexor: 0/0 Finger grasping: 0/0 Hip adductor: 1/2 Knee flexor: 3/3 Ankle plantarflexor: 1+/1+ Pathologic Rel (R/L) Babinski: -/- Hoffmann: -/- Gegenhalten paratonia(+, right arm), tremor(+ right arm and fingers), bradykinesia(+/- bilateral arms) [Sensation] - Pinprick/light touch: intact - Proprioception: intact [Sphincter Function] Defecation: partially continent, dependent on dulcolax and diaper Urination: partially continent, dependent on diaper [Functional Status] Balance: Sitting (static/dynamic): fair/poor Standing (static/dynamic): NA/NA Function: Rolling: moderate assistance Sitting up: moderate assistance Standing up: NA Transfer: moderate assistance Ambulation: NA Slope/Stairs: NA Func Ambulation Classification (FAC): 0 [Barthel Index] 5/100 Feeding: 5/10, Transfer: 0/15, Grooming: 0/5, Toilet: 0/10, Bathing: 0/5, Walking: 0/15, S 0/10, Dressing: 0/10, Bowel: 0/10, Bladder: 0/10 [Modified Rankin Scale (mRS)] 5

Muscle power



as above

Laboratory Data

2026/04/30(血漿) Na=137 K=3.4(L) GOT=21 GPT=36(H) CRP=2.747(H)

Procalcitonin=0.084(H)

2026/04/30(血液) Hb=11.4(L) Ht=36.0(L) RBC=4.63 RDW=20.0(H) WBC=4.66

人工關片=No Neutrophil Seg.=70.4 Lymphocyte=16.7(L) Monocyte=11.4(H)

Eosinophil=0.4 Basophil=1.1(H) Absolute Neutrophil count=3280 MCV=77.8

(L) MCH=24.6(L) MCHC=31.7 PLT=187

2026/04/27(尿 (中段尿)) Appearance=Yellow/Turbid Microscopic RBC=50-99

(*) Microscopic WBC=6-9(*) Ep.cell=0-5 Cast(1)- Crystal (1)-

Bacteria=2+(*) Glucose=- Bilirubin=- Ketone Body=- Sp.gr=1.012 OB=2+

(*) PH=7.0 Protein=+- Urobilinogen=1.0 Nitrite=- Leukocyte=1+(*)

2026/04/27(血漿) Na=137 K=3.2(L) Glucose=104 BUN=11 GOT=22 GPT=71

(H) Creatinine=0.32(L) eGFR(MDRD計算公式, 僅供參考)=218.58 eGFR(CKD-EPI計

算公式, 僅供參考)=127.16 CRP=6.445(H)

2026/04/27(血液) Hb=11.4(L) Ht=34.8(L) RBC=4.57 RDW=21.2(H) WBC=8.81

人工關片=No Neutrophil Seg.=73.3 Lymphocyte=16.1(L) Monocyte=10.1(H)

Eosinophil=0.0 Basophil=0.5 Absolute Neutrophil count=6460 MCV=76.1(L)

MCH=24.9(L) MCHC=32.8 PLT=195

Images

《鼻喉內視鏡檢查》

2026/04/21 :

下咽及喉部(Hypopharynx/Larynx)

1. 聲帶關閉不全(Incomplete Closure Of Vocal Cord)
2. 聲帶萎縮(Vocal Cords Atrophy)
3. 環狀軟骨後區腫脹(Post Cricoid Area Edema)
4. symmetric vocal movement
bilateral vocal cord atrophy

《磁共振造影檢查(MRI)》

2026/04/15 :

Finding : (檢查完成時間顯示在影像上)

MRI--HEAD&NECK : Routine Brain(CEMR) : performed WITHOUT & WITH intravenous contrast enhancement.(顯影劑:Gadoteric Acid

FINDINGS

- *Multifocal patchy cortical T1 hyperintensity of bilateral fronto-parietal gyrus, left basal ganglion and left temporal
 - *Multiple restricted diffusion in the bilateral fronto-parietal cerebral sulci, cistern of the laminae terminalis and s
 - *Enlarged cisterna magna
 - *MRA shows: suspect spasm of bilateral ACAs.
- Impression: 1.Suspect multifocal patchy petechial hemorrhage of bilateral fronto-parietal gyrus, left basal ganglion an
- 2.Suspect residual hematoma in the bilateral fronto-parietal cerebral sulci, cistern of the laminae terminalis and supe
- 3.Enlarged cisterna magna
- 4.Suspect spasm of bilateral ACAs.

《電腦斷層掃描(CT)》

2026/03/26 :

Finding : (檢查完成時間顯示在影像上)

CT--HEAD & NECK :Routine Brain(CECT) : performed WITHOUT & WITH intravenous contrast enhancement.(顯影劑:Ioversol(Optir

FINDINGS:

INTRACRANIAL SPACE:

- * S/P Acom aneurysm coiling. S/P EVD with ICP monitoring insertion.
- * Mild SAH in bilateral F-P sulci.
- * Mild IVH in lateral ventricles.
- * Small anterior interhemispheric resolving hematoma is suspected (SE/IM:201/13).

CTA :

- * Diffuse luminal narrowing of bilateral ACA without obstruction.
- * No definite residual aneurysm in bilateral major intracranial arteries.

SKULL and other findings within the visible scanning fields:

- * S/P right craniotomy.

Impression: IMPRESSIONS:

- * S/P Acom aneurysm coiling. S/P EVD with ICP monitoring insertion.
- * Mild bilateral F-P SAH and lateral ventricle IVH.
- * Small anterior interhemispheric resolving hematoma is suspected.
- * Spastic appearance of bilateral ACA is suspected. Suggest follow up.

《電腦斷層掃描(CT)》

2026/03/15 :

Finding : (檢查完成時間顯示在影像上)

CT--HEAD & NECK :Routine Brain(CECT) : performed WITHOUT & WITH intravenous contrast enhancement.(顯影劑:Ioversol(Optir

FINDINGS:

INTRACRANIAL : Extensive SAH. Associated hydrocephalus. A 4.0mm A-com aneurysm is noted.

SKULL AND SKULL BASE: n.p.

EXTRACRANIAL (including PARANASAL SINUSES, ORBITAL CAVITIES, and SCALP) : n.p.

Impression: Impression:

A-com aneurysm rupture with extensive SAH and hydrocephalus.

《電腦斷層掃描(CT)》

2026/03/15 :

Finding: Conscious change in this evening.

(檢查完成時間顯示在影像上)

CT--HEAD & NECK :Routine Brain : performed WITHOUT intravenous contrast enhancement.

FINDINGS:

INTRACRANIAL : Extensive SAH. Associated hydrocephalus.

SKULL AND SKULL BASE: n.p.

EXTRACRANIAL (including PARANASAL SINUSES, ORBITAL CAVITIES, and SCALP) : n.p.

Impression: Impression:

Extensive SAH. Associated hydrocephalus.

Impression

Anterior communicating artery aneurysm rupture with extensive SAH (subarachnoid hemorrhage) and hydrocephalus s/p (status post) 1. EVD (Extraventricular Drainage) with ICP (intracranial pressure) monitoring insertion (via right Kocher's point) (Codman ICP (intracranial pressure)) on 2025/03/15; 2. Trans-arterial embolization of anterior communicating artery aneurysm (simple coiling), sonography guided femoral puncture + DSA (Digital Subtraction Angiograph) Bilateral ICA (internal carotid artery), Left VA (vertebral artery), Right femoral artery on 2026/03/15, with bilateral hemiplegia, post-stroke spasticity, and cognitive impairment

Hypertension

Plan

Bilateral hemiplegia

- Arrange PT, OT, ST programs
- PT: passive ROM, stretching, facilitation, muscle strengthening, endurance training, therapeutic exercise, balance training, ambulation training,
- OT: functional training, transfer training, sitting and standing balance training, ADL training
- ST: auditory comprehension, verbal production, augmentative communication, reading training, writing training, tactile stimulation, oral training, swallowing training

Bilateral leg spasticity

- Arrange CT to rule out hydrocephalus
- If hydrocephalus consider ventriculoperitoneal shunt
- Tolperisone titrate to maximal dose, then bridge to Befon (no tolperisone in our hospital) based on her spasticity
- Consider ESWT for bilateral hamstring muscles
- Consider cashed botulinum toxin injections.

Paramedical

- 巴 (X) 殘 (V) 長照(V) 低收(X)
- Key person: husband
- Caregiver: husband (看護跑掉)
- Schedule: 入院: 07/08 4W: 08/05
- Disposition: transfer

Rehab. goal

- Minimal assistance in all transfers
- Moderate assistance with household ambulation (>10 meters) with quadricane or walker only if spasticity improved
- Simple ADLs including left hand feeding after setup and partial independence with dressing, toileting and bathing
- Achieve functional communication
- Identify caregivers to provide supervision after discharge for care skills

-住院的主要目的：復健訓練與疾病治療

-治療及處置計畫(如侵入性處置)：物理治療、職能治療、語言治療

主治醫生意見: Arrange intensive rehabilitation programs; Arrange brain CT for suspect delayed hydrocephalus

Recorded by:

Resident: 吳一宏

Attending: 黃雅鈴